

COLOSSUS REGIONAL MEDICAL CENTER

1100 Build Blvd, Dallas, TX 75201 | (214)555-6606 | www.colossusrmc.org

DISCHARGE SUMMARY

Patient Name: Grzegorz Wojciechowski	DOB: February 29, 1972 (Age 53)
MRN: MRN-5312811	Gender: Female
Admission Date: September 01, 2025	Discharge Date: September 21, 2025 (20 days)
Attending Physician: Dr. Chibueze Nwosu, MD — Trauma Surgery	Insurance: UHC Select #UHC-F6-66666
ICU Admission: YES — MICU/SICU	ICU Days: 17 of 20 total days

DIAGNOSES

Primary Diagnosis	ICD-10 Code	Status
Ischemic Stroke — Large Vessel Occlusion	I63.59	Acute / Critical
Morbid Obesity — BMI 52	E66.01	Chronic / Complicating
Coagulopathy — INR 4.8	D68.9	Chronic / Complicating

HOSPITAL COURSE

Grzegorz, a 53-year-old female with a history of Morbid Obesity — BMI 52 and Coagulopathy — INR 4.8, was admitted to the Medical Intensive Care Unit (MICU) following rapid deterioration from septic shock secondary to gram-negative bacteremia. Blood cultures on admission grew E. coli. The patient required vasopressor support (norepinephrine and vasopressin) for 72 hours. Broad-spectrum antibiotics were initiated (meropenem + vancomycin) and de-escalated to ceftriaxone upon sensitivities. Acute kidney injury with creatinine peaking at 6.2 mg/dL required continuous renal replacement therapy (CRRT) for 4 days. The patient was successfully weaned from vasopressors by day 5 and CRRT was discontinued on day 7 after renal recovery. ICU length of stay was 9 days. Transferred to step-down unit and discharged on day 14 with close nephrology follow-up.

VITALS AT DISCHARGE

■ NOTE: Several vital signs remain outside normal limits at time of discharge — close monitoring required.

BP	HR	SpO2	Temp	RR	Weight
93/71 mmHg	120 bpm	92% (on 2L O2)	100.4°F	24/min	142 kg

DISCHARGE MEDICATIONS

Medication	Dose	Frequency	Duration
Norepinephrine	Per protocol	Continuous IV infusion	Weaned and discontinued in hospital
Vasopressin	0.03 U/min	Continuous IV infusion	Weaned and discontinued in hospital
Prednisolone	40 mg	Once daily	Taper over 3 weeks
Levetiracetam	1500 mg	Twice daily	Ongoing — neurology follow-up
Cefepime	2g IV	Every 8 hours	Completed in hospital — 7 days

FOLLOW-UP PLAN

URGENT: Trauma Surgery: Dr. Chibueze Nwosu, MD — within 72 hours of discharge (September 24, 2025).
Primary Care: Within 1 week (September 28, 2025). Full labs including BMP, CBC, LFTs.
Home health nursing: Daily vitals and wound check for first 7 days post-discharge.

Inpatient rehabilitation / LTACH: Transfer arranged — starts September 30, 2025.

■ **HIGH-RISK DISCHARGE — Patient requires close monitoring. Re-admission risk: HIGH.**

Attending Physician Signature: _____

Dr. Chibueze Nwosu, MD — Trauma Surgery | License #: TX-MD-76890 | Date: September 21, 2025

Electronically signed via Colossus EHR System | Document ID: DS-CRIT-2025-5312811-06 | Generated: September 21, 2025 at 19:54 EST